

Bureau of Export Administration, Commerce

§ 700.52

Commerce. A sample Form BXA-999 is attached at Appendix I.

[49 FR 30414, July 30, 1984; 49 FR 50171, Dec. 27, 1984. Redesignated at 54 FR 601, Jan. 9, 1989, as amended at 63 FR 31924, June 11, 1998]

§ 700.51 Requests for priority rating authority.

(a) If a rated order is likely to be delayed because a person is unable to obtain items not normally rated under this regulation, the person may request the authority to use a priority rating in ordering the needed items. Examples of items for which priority ratings can be authorized include:

(1) Production or construction equipment;

(2) Computers when not used as production items; and

(3) Expansion, rebuilding or replacing plant facilities.

(b) *Rating authority for production or construction equipment.* (1) A request for priority rating authority for production or construction equipment must be submitted to the appropriate Delegate Agency. The Delegate Agency may establish particular forms to be used for these requests (e.g., Department of Defense Form DD 691.)

(2) When the use of a priority rating is authorized for the procurement of production or construction equipment, a rated order may be used either to purchase or to lease such equipment. However, in the latter case, the equipment may be leased only from a person engaged in the business of leasing such equipment or from a person willing to lease rather than sell.

(c) *Rating authority in advance of a rated prime contract.* (1) In certain cases and upon specific request, Commerce, in order to promote the national defense, may authorize a person to place a priority rating on an order to a supplier in advance of the issuance of a rated prime contract. In these instances, the person requesting advance rating authority must obtain sponsorship of the request from the appropriate Delegate Agency. The person shall also assume any business risk associated with the placing of rated orders if these orders have to be cancelled in the event the rated prime contract is not issued.

(2) The person must state the following in the request:

It is understood that the authorization of a priority rating in advance of our receiving a rated prime contract from a Delegate Agency and our use of that priority rating with our suppliers in no way commits the Delegate Agency, the Department of Commerce or any other government agency to enter into a contract or order or to expend funds. Further, we understand that the Federal Government shall not be liable for any cancellation charges, termination costs, or other damages that may accrue if a rated prime contract is not eventually placed and, as a result, we must subsequently cancel orders placed with the use of the priority rating authorized as a result of this request.

(3) In reviewing requests for rating authority in advance of a rated prime contract, Commerce will consider, among other things, the following criteria:

(i) The probability that the prime contract will be awarded;

(ii) The impact of the resulting rated orders on suppliers and on other authorized programs;

(iii) Whether the contractor is the sole source;

(iv) Whether the item being produced has a long lead time;

(v) The political sensitivity of the project; and

(vi) The time period for which the rating is being requested.

(4) Commerce may require periodic reports on the use of the rating authority granted under paragraph (c) of this section.

(5) If a rated prime contract is not issued, the person shall promptly notify all suppliers who have received rated orders pursuant to the advanced rating authority that the priority rating on those orders is cancelled.

§ 700.52 Examples of assistance.

(a) While special priorities assistance may be provided for any reason in support of this regulation, it is usually provided in situations where:

(1) A person is experiencing difficulty in obtaining delivery against a rated order by the required delivery date; or

(2) A person cannot locate a supplier for an item needed to fill a rated order.

(b) Other examples of special priorities assistance include:

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- (1) Ensuring that rated orders receive preferential treatment by suppliers;
- (2) Resolving production or delivery conflicts between various rated orders;
- (3) Assisting in placing rated orders with suppliers;
- (4) Verifying the urgency of rated orders; and
- (5) Determining the validity of rated orders.

§ 700.53 Criteria for assistance.

Requests for special priorities assistance should be timely, *i.e.*, the request has been submitted promptly and enough time exists for the Delegate Agency or Commerce to effect a meaningful resolution to the problem, and must establish that:

- (a) There is an urgent need for the item; and
- (b) The applicant has made a reasonable effort to resolve the problem.

§ 700.54 Instances where assistance will not be provided.

Special priorities assistance is provided at the discretion of the Delegate Agencies and Commerce when it is determined that such assistance is warranted to meet the objectives of this regulation. Examples where assistance may not be provided include situations when a person is attempting to:

- (a) Secure a price advantage;
- (b) Obtain delivery prior to the time required to fill a rated order;
- (c) Gain competitive advantage;
- (d) Disrupt an industry apportionment program in a manner designed to provide a person with an unwarranted share of scarce items; or
- (e) Overcome a supplier's regularly established terms of sale or conditions of doing business.

[49 FR 30414, July 30, 1984. Redesignated at 54 FR 601, Jan. 9, 1989, as amended at 63 FR 31924, June 11, 1998]

§ 700.55 Assistance programs with Canada and other nations.

(a) To promote military assistance to foreign nations, this section provides for authorizing priority ratings to persons in Canada and in other foreign nations to obtain items in the United States in support of approved programs. Although priority ratings have no legal authority outside of the

United States, this section also provides information on how persons in the United States may obtain informal assistance in Canada.

(b) *Canada.* (1) The joint U.S.-Canadian military arrangements for the defense of North America and the integrated nature of their defense industries as set forth in the *U.S.-Canadian Statement of Principles for Economic Cooperation* (October 26, 1950) require close coordination and the establishment of a means to provide mutual assistance to the defense industries located in both countries.

(2) The Department of Commerce coordinates with the Canadian Public Works and Government Services Canada on all matters of mutual concern relating to the administration of this regulation. A copy of the Memorandum of Understanding between the two departments is provided at appendix IV.

(3) Any person in the United States ordering defense items in Canada should inform the Canadian supplier that the items being ordered are to be used to fill a rated order. The Canadian supplier should be informed that if production materials are needed from the United States by the supplier or the supplier's vendor to fill the order, they should contact the Canadian Public Works and Government Services Canada for authority to place rated orders in the United States.

(4) Any person in Canada producing defense items for the Canadian government may also obtain priority rating authority for items to be purchased in the United States by applying to the Canadian Public Works and Government Services Canada in accordance with procedures specified by that Department.

(5) Persons in Canada needing special priorities assistance in obtaining defense items in the United States may apply for such assistance to the Canadian Public Works and Government Services Canada. Public Works and Government Services Canada will forward appropriate requests to Commerce.

(6) Any person in the United States requiring assistance in obtaining items in Canada must submit a request through the Delegate Agency to Commerce on Form BXA-999. Commerce

*Peer Review Framework for Predictive Analytics
in Humanitarian Response*

MODEL REPORT: Cooper/Smith population mobility model

November 2021

OCHA CENTRE FOR HUMANITARIAN DATA



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COOPER / SMITH



Model Report: Cooper/Smith population mobility model

1. Background

In 2020 the technical assistance firm [Cooper/Smith](#) developed a population mobility model in Malawi with the support of the [Digital Impact Alliance](#) (DIAL) and the Malawi Ministry of Health (MoH). The mobility model provides insight on population localization and movement patterns. It enables the MoH and local stakeholders to have a near real-time view on where the population is, providing a much-needed complement to the census.

Between November 2020 and January 2021, the OCHA Centre for Humanitarian Data (the Centre) engaged with technical and ethical experts¹ to conduct an independent review of Cooper/Smith's model in Malawi, according to the Centre's [Peer Review Framework for Predictive Analytics in Humanitarian Response](#). Between February and November 2021 The Centre worked with Cooper/Smith and our reviewers to finalise the recommendations presented below.

2. Main Findings and Recommendations

The population mobility model provides insight on population localization and movement patterns, enabling the Ministry of Health (MoH) of Malawi and local stakeholders to have an almost real-time view on where the population is. It signals when there is a large gathering that is two standard deviations from the mean number of subscribers in that location (traditional authority or district level) over the last 30-days.

The primary user of this model is the Government of Malawi's Public Health Emergency Operations Center (PHEOC). The model has an automated signal that will send emails to PHEOC members and the PHEOC's Incident Management System, which logs, triages, and actions reported incidents.

¹ For this project we collaborated with technical experts from UN Global Pulse and ethics experts from the Centre for Humanitarian Data.

You can find all the documentation regarding the model, its application and the review process at the following links:

- The [Model Card](#) describes version 2 of the model developed in Q2 2020.
- The [Model Evaluation Matrix](#) was completed in November 2020 by experts from the OCHA Centre for Humanitarian Data and UN Global Pulse.
- The [Implementation Plan](#) was completed in November 2020. It summarizes the concrete actions that the model will trigger or inform.
- The [Ethical Matrix](#) aims to identify all stakeholders and potential issues regarding the intended use of the model. The Ethical Matrix was completed In January 2021.

A summary of the main findings and recommendations is provided below.

2.1 Technical Review

Model development and documentation

- The Centre recommended clearly distinguishing the three modeling tasks and clarifying which methods are being used to perform each of the three tasks when presenting the model to potential stakeholders and end users.
 - This recommendation has been considered and met by Cooper/Smith, as detailed in the [Model Evaluation Matrix](#) and the [Model Documentation](#).

Model Evaluation

- Depending on data availability, we recommend performing a quantitative analysis of model performance.
- Additional datasets used to independently validate the model (Google Mobility; observed events counts; case counts) all have limitations. We recommend that Cooper/Smith should continue seeking alternative sources to compare estimates and understand correlation/discrepancies between them.
- Cooper/Smith should continue to operate a feedback process from model end users and health care workers on the ground, and we recommend enlarging it to community members and leaders. Validating findings from the model with a larger group of actors (e.g. community health workers, community leaders) would ensure greater accuracy and usefulness.

2.2 Ethical Review

False Positives and Negatives

When the model produces a false alarm this is considered a 'false positive'. When the model misses an event this is considered a 'false negative'. False positives and negatives can cause costly interventions or lack thereof, for example sending personnel or resources to an area where they were not urgently needed.

- Cooper/Smith has identified a risk mitigation strategy that includes an ongoing feedback loop with primary stakeholders about how to handle false alarms, helping decrease the level of concern. The Centre recommends continuing to follow this strategy closely to prevent any potential issues from materializing.

Overreaction

Overreaction is a decision informed by the model output that leads to action that is too impactful compared to the need that should be addressed.

- Cooper/Smith has already established a decision process and protocol for the use of the model to mitigate risk related to Overreaction. In future applications of the model, we suggest creating a limited list of decisions that may be informed by the model output, in order to ensure the output is not used to inform decisions leading to action that is incompatible with applicable guidance and ethical principles, including the Humanitarian Principles.

Inaction

When no action is taken based on the model output this is considered 'inaction'.

- The Centre recommends that the risk reduction strategy identified in the [Model card](#) be followed closely and its implementation be monitored. In future model deployments, the Centre recommends the model only be deployed when there is an established plan for action based on its outputs.

Privacy / Rights Infringement

When data or information is managed or exposed in ways that infringe upon human rights, this is considered a 'privacy / rights infringement'.

- The data currently used in the model is anonymized and aggregated. Should the model use personal or otherwise sensitive data in future applications, we suggest its management to be checked by legal / data protection experts.

Gaming

When stakeholders can act according to the algorithm's rules to achieve an intended outcome, this is considered 'gaming'.

- The Centre recommends that any significant changes in model behavior be monitored closely in order to assess whether they might be caused by gaming. Sudden changes that affect the outcome of the model could indicate that the target population is adjusting its behavior to avoid gatherings from being detected.

22. Device as claimed in claim 21 wherein said retention means includes a plurality of spacers, said spacers being substantially completely inert to said liquid crystal material.

23. A liquid crystal optical switching apparatus; said apparatus comprising:

a first liquid crystal optical switching device having first and second transparent members, and said first and second members having a base surface associated therewith, said base surfaces being spaced apart by liquid crystal material, said first transparent member having first input and output ports associated therewith, said second transparent member having a second output port associated therewith;

a second liquid crystal optical switching device, having third and fourth transparent members, each said third and fourth members having a base surface associated therewith, said base surfaces spaced apart by liquid crystal material, said third transparent member having third input and output ports associated therewith, said fourth transparent member having fourth input and output ports associated therewith;

means, associated with each said input and output port having means, extending into said transparent member associated therewith, for receiving an optical fiber;

means for sustaining an electromagnetic field in said liquid crystal material;

a first optical path between said first output port and said third input port; and

a second optical path between said second output port and said fourth input port.

24. Apparatus as claimed in claim 23 wherein said second transparent member further comprises:

a second input port.

25. Apparatus as claimed in claim 24 wherein said optical fiber receiving means includes a blind hole extending into said members associated therewith at about the critical angle.

26. Apparatus as claimed in claim 25 further comprising:

means, associated with each said port and disposed in each said blind hole, for collimating a light beam passing therethrough.

27. Apparatus as claimed in claim 23 wherein said electromagnetic field sustaining means includes first, second, third and fourth electrodes overlying said base surfaces.

28. Apparatus as claimed in claim 27 further comprising

means for matching the refractive indices of each said electrode and said liquid crystal material.

29. Apparatus as claimed in claim 27 further comprising:

means for matching the refractive indices of said electrodes and said base surfaces associated therewith.

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CHAPTER 23-16

LICENSING MEDICAL HOSPITALS

23-16-01. Licensure of medical hospitals and state hospitals. After July 1, 1947, no person, partnership, association, corporation, limited liability company, county or municipal corporation, or agency thereof, which maintains and operates organized facilities for the diagnosis, treatment, or care of two or more nonrelated persons suffering from illness, injury, or deformity or where obstetrical or other care is rendered over a period exceeding twenty-four hours, may be established, conducted, or maintained in the state of North Dakota without obtaining annually a license therefor in the manner hereinafter provided in sections 23-16-02 and 23-16-03. Chiropractic hospitals, sanatoriums, and hospitals such as those for unmarried mothers maintained and operated by the department of human services are not required to obtain a license under this chapter.

In the case of emergency or transfer beds attached to and forming a part of a licensed medical doctor's office, the state department of health has the right of inspection, but no license may be required under the provisions of this chapter when the number of such beds does not exceed four.

23-16-01.1. Moratorium on expansion of long-term care bed capacity.

1. Notwithstanding sections 23-16-06 and 23-16-10, except when a facility reverts basic care beds to nursing facility beds, nursing facility beds may not be added to the state's licensed bed capacity during the period between August 1, 2009, and July 31, 2011. A nursing facility may not convert licensed nursing bed capacity to basic care bed capacity or convert basic care beds back to nursing facility beds more than one time in a twelve-month period if the beds have been licensed as basic care.
2. Transfers of beds from one facility to another entity is permitted. Transferred nursing facility beds must become licensed within forty-eight months of transfer. Nursing facility beds transferred before August 1, 2005, which are awaiting nursing facility licensure, may be converted to basic care licensure.
3. A nursing facility may convert licensed nursing facility bed capacity to basic care. If the converted beds remain in the same facility and are not transferred, the beds may revert to nursing facility status after one year of licensure as basic care beds.
4. Nursing facility beds that are converted to basic care may be transferred as basic care beds. However, upon the transfer, the basic care beds may not be relicensed as nursing facility beds.
5. If an Indian tribe acquires nursing facility beds, the tribal facility must meet state licensing requirements for those beds within forty-eight months of acquisition. A tribal facility may seek to participate in the medical assistance programs. Medical assistance payments may only be made to a medicaid certified tribal facility that agrees to participate and adhere to all federal and state requirements of the medical assistance program, including participation, screening, ratesetting, and licensing requirements.

23-16-02. Existing medical hospitals. Institutions subject to this chapter which are already in operation at the time of enactment of this chapter must be given a reasonable time, not to exceed one year from the date of the enactment of this chapter, within which to comply with the rules, regulations, and minimum standards provided for herein.

23-16-03. Application for license - License fee. Applicants for license shall file applications under oath with the state department of health upon forms prescribed. Applications must be signed by the owner, or in the case of a corporation by two of its officers, or in the case

of a county or municipal unit by the head of the governmental department having jurisdiction over it. Applications must set forth the full name and address of the owner of the institution for which license is sought, the names of the persons in control thereof, and such additional information as the state department of health may require, including affirmative evidence of ability to comply with such minimum standards, rules, and regulations as may be lawfully prescribed pursuant to this section. An application for a license for facilities not owned by the state or its political subdivisions must be accompanied by the following fees:

1. For each licensed acute care bed, ten dollars.
2. For each licensed skill care bed, ten dollars.

License fees collected pursuant to this section must be deposited in the state department of health services operating fund in the state treasury and any expenditure from the fund is subject to appropriation by the legislative assembly.

23-16-04. Licenses. Licenses issued hereunder expire one year after date of issuance or upon such uniform dates annually, as the health council may prescribe by rule. Licenses must be issued only for the premises and persons named in the application and are not transferable or assignable. Licenses must be posted in a conspicuous place on the licensed premises.

23-16-05. Inspections, consultations, and approval of plans. The state department of health shall make or cause to be made such inspections as may be prescribed by regulation. The health council may prescribe by regulations that any licensee or prospective applicant desiring to make a substantial alteration or addition to its facilities or to construct new facilities shall, before commencing such alteration, addition, or new construction, submit plans and specifications therefor to the state department of health for preliminary inspection, recommendation, and approval.

23-16-06. Authority to issue, deny, suspend, or revoke licenses. The state department of health shall issue licenses for the operation of institutions subject to this chapter which are found to comply with the provisions of this chapter and such regulations as are lawfully promulgated by the health council. The state health officer with the approval of the health council may, after a hearing, suspend or revoke licenses issued hereunder on any of the following grounds:

1. Violation of any of the provisions of this chapter or the rules and regulations promulgated pursuant thereto.
2. Permitting, aiding, or abetting the commission of any unlawful act.
3. Conduct or practices detrimental to the health or safety of patients and employees of said institutions; provided that this provision may not be construed to have any reference to practices authorized by law; and provided further that no license may be suspended or revoked for any trivial violation.

No application for a license may be denied, or any licenses suspended or revoked, except after a hearing before the health council held pursuant to written notice to the applicant or licensee, served by registered or certified mail, which notice must concisely state the grounds for such denial or for such proposed suspension or revocation and must fix the time and place of hearing which may not be less than thirty days after the date of the mailing of such notice. After such hearing, the council shall make an order, either denying the application for license or granting the same, or suspending or revoking such license, or dismissing the proceedings to suspend or revoke as the merits of the case warrant. The council shall send a copy of its order to the applicant or licensee by registered or certified mail, which must contain its findings and conclusions, and such order, except an order of dismissal, becomes final thirty days after the date of mailing unless the applicant or licensee appeals therefrom in the manner provided by section 23-16-10.

23-16-07. Not applicable to certain laws. This chapter may not be construed in any way to restrict or modify any law pertaining to the placement and adoption of children or the care of unmarried mothers.

23-16-08. Offering or advertising to dispose of infants prohibited. No hospital providing maternity care may in any way offer to dispose of any child or advertise that it will give children for adoption or hold itself out, directly or indirectly, as being able to dispose of children, however, such hospitals may inform an unmarried mother of child-placing agencies licensed by the department of human services.

23-16-08.1. Access to pharmacist. Irrespective of the type of distribution system used, no person may refuse to allow a resident of a nursing home, as defined in subsection 3 of section 43-34-01, to choose a pharmacist of the resident's choice for the compounding and dispensing of drugs pursuant to chapter 43-15.

23-16-09. Information confidential. Information other than reports relating to vital statistics received by the state department of health through inspection or otherwise, authorized under this chapter are confidential and may not be disclosed publicly except in a proceeding involving the question of license. No agent of the state department of health or of any board of health, may disclose individually identifiable health information of such an institution obtained in the course of a survey or inspection except in a judicial or administrative proceeding in response to an order of a court or administrative tribunal.

23-16-10. Appeal. An appeal may be taken to the district court from any order of the state health officer or health council denying an application for a license to operate a medical hospital or related institution, or suspending or revoking a license, or from any order denying an application for a construction project. Any such appeal must be taken in the manner provided in chapter 28-32.

23-16-11. Penalties.

1. Any person establishing, conducting, managing, or operating any institution subject to this chapter, without first obtaining a license as required by this chapter, or who violates any of the provisions of this chapter is guilty of an infraction.
2. In addition to any criminal sanctions that may be imposed pursuant to law, any person maintaining or operating a nursing facility licensed by the department who is found guilty of knowingly violating any provision of this title or any rules adopted under this title, or any person maintaining or operating a nursing facility found to have deficiencies during a survey of the nursing facility, may be assessed a civil penalty not to exceed one thousand dollars for each violation and for each day the violation continues plus interest and any costs incurred by the department to enforce this penalty. This civil penalty may be imposed by a court in a civil proceeding or by the state health officer through an administrative hearing under chapter 28-32. If a civil penalty levied by the department after an administrative hearing is not paid within thirty days after a final determination that a civil penalty is owed, unless the determination of a civil penalty is appealed to a district court, the civil penalty and any costs incurred by the department to enforce the penalty may be withheld from payments due to the person or nursing facility from the department of human services. Any funds received as penalties must be applied to protect residents of the nursing facility, to relocate residents, to maintain operation of the nursing facility, and to reimburse residents for loss of personal funds.

23-16-12. Injunction. The state department of health, in accordance with the laws of the state governing injunctions and other process, may maintain an action in the name of the state against any person, partnership, association, corporation, or limited liability company for establishing, conducting, managing, or operating any hospital within the meaning of the chapter without first having a license therefor as herein provided or without first obtaining from the state

department of health written approval of plans and specifications for major alterations of, additions to, or construction of health facilities.

23-16-13. Appropriation. Repealed by omission from this code.

23-16-14. Participation in abortion - Not mandatory. No hospital, physician, nurse, hospital employee, nor any other person is under any duty, by law or contract, nor may such hospital or person in any circumstances be required to participate in the performance of an abortion, if such hospital or person objects to such abortion. No such person or institution may be discriminated against because the person or institution so objects.

23-16-15. Umbilical cord blood donation.

1. Unless it is medically inadvisable, a hospital shall allow a pregnant patient to arrange for the blood extracted from the umbilical cord of the patient's newborn child to be donated to a public cord blood bank. A patient who agrees to donate cord blood to a public cord blood bank may not be charged for the costs of collecting, storing, or transporting the cord blood.
2. A hospital is not required to collect cord blood if in the professional judgment of a licensed physician the collection of the cord blood would threaten the health of the mother or newborn child. This section does not require a hospital or hospital employee, including a physician, nurse, or other medical staff, to collect cord blood if the collection of cord blood conflicts with the bona fide religious practices and beliefs of the hospital or hospital employee. This section does not require a hospital to arrange for the donation of blood extracted from umbilical cords.

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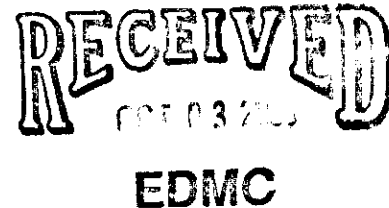
Waste Site Reclassification Form

Date Submitted: 5/12/2000	Operable Unit(s): 200-TW-2	Control Number: 2000-049
Originator: Charlie Shipler	Waste Site ID: 200-W-48	
Phone: 372-2644	Type of Reclassification Action:	
	Rejected ☒ Closed-Out ☐ No Action ☐	

This form documents agreement among the parties listed below authorizing classification of the subject unit as rejected, closed-out, or no action and authorizing backfill of the site, if appropriate. Final removal from the NPL of no action or closed-out sites will occur at a future date.

Description of current waste site condition:

The site was a self-contained conex box with a spill berm, reported to be approximately 25 meters (75 feet) west of the 241-TX Tank Farm. The box has been removed.


 RECEIVED
 OCT 03 2000
 EDMC

Basis for reclassification:

WAC 173-303-630(10) requires that at closure, all dangerous waste and dangerous waste residues be removed from the containment system. However, there are no documentation requirements for this closure, and thus no referenceable records for older pads. For many of these older pads, the exact locations were not recorded and cannot be reliably re-examined at this time. By analysis of the spill response standard operating procedure and interviews with the spill response coordinator, documentation of closure of these 90 Day Storage Areas can be recreated.

According to J. Culmer (July 31, 2000), spill response and reporting coordinator, there have been no releases to the environment from 90-Day pads. The HNF-PRO-453 Spill and Release Reporting procedure covers cleaning up releases and entering release sites, where releases cannot be immediately cleaned up due to the location of the release or the decision made to clean up the site at a later date, as part of the Operable Unit. If the release is to be cleaned up as part of the Operable Unit, then it is required that the release site and material released be added to an existing Operable Unit WIDS site or a new WIDS site created and entered into the data base. Mr. Culmer is not aware of any spills or releases onto 90 Day Storage Pads, that have not been cleaned up immediately, waste managed properly and then disposed of per state and federal requirements. All releases have been handled per the HNF-PRO-453 Spill and Release Reporting procedure. The releases onto 90 Day Storage Pads that had been reported internally to the spill response office were small leaks from containers that had been cleaned up; none of these releases migrated out of the connex box (90 Day Pad) and into the soil column or environment. Thus, all evidence

<u>HAROLD J. STAFFORD JR.</u>	<u>Harold J. Stafford Jr.</u>	<u>8/21/00</u>
DOE Project Manager	Signature	Date
<u>Wayne Soper</u>	<u>Wayne Soper</u>	<u>9-6-00</u>
Ecology Project Manager	Signature	Date
_____ EPA Project Manager	Signature	Date